



FOR PUBLICATION

DERBYSHIRE COUNTY COUNCIL

REPORT TO CABINET MEMBER FOR HEALTH AND COMMUNITIES

22 January 2024

Report of the Director - Public Health

Strengthen Digital Support in Derbyshire
(Cabinet Member for Health and Communities)

1. Divisions Affected

1.1 County-wide.

2. Key Decision

2.1 This is not a Key Decision.

Purpose

2.2 The Cabinet Member for Health and Communities is asked to approve the award of grants to:

- a) Rural Action Derbyshire (RAD) to coordinate the Digital Support Derbyshire Network (DSDN) for 2 years, subject to satisfactory performance, at a cost of £0.050m per annum and a total value of £0.100m**
- b) Voluntary, Community & Social Enterprise (VCSE) partners to deliver work that strengthens community based digital inclusion. The Digital Support Derbyshire Network will administer the governance, approval and issuing of small grants to community partners within the network, at a cost of £0.050m per annum for a maximum 2 years at a total value of £0.100m.**

3. Information and Analysis

- 3.1 There is a strong correlation between digital exclusion, financial exclusion, and social exclusion. Covid-19 led to a widening of this digital divide, as more products and services moved online and became 'digital by default'. Additionally, those facing digital exclusion are more likely to have other challenges that make it difficult for them to access information and engage with online services such as adults with a registered disability or long-term health condition and/or on low incomes.
- 3.2 Increasing opportunities for digital inclusion is essential to reduce health inequalities and helps tackle financial and social exclusion by (1) empowering residents to develop and improve digital skills, ensuring everyone is connected, and (2) embedding digital inclusion in the everyday work of organisations, as well as ensuring that this work is sustainable.
- 3.3 Recognising this, in June 2021, the Public Health Investment Panel supported the proposed investment from the public health reserve to develop a public health approach to address emerging digital inclusion issues.
- 3.4 Following a robust procurement exercise, Citizens Online were awarded the contract and commenced work in April 2022. Citizens Online collaborated with 655 stakeholders to understand digital exclusion risk factors in Derbyshire and produced a draft public health digital inclusion strategy with a set of recommendations.
- 3.5 Whilst the strategy was being developed, VCSE partners across Derbyshire were also forming the DSDN. Work has been coordinated to date by RAD, utilising one-off short-term funding from NHS Charities Together to deliver community based digital services in localities across Derbyshire.
- 3.6 To maximise resources and reduce duplication, Citizens Online and the DSDN collaborated extensively to inform the emerging strategy and drive the agenda forward. A significant mapping exercise to identify existing digital inclusion provision in Derbyshire contributed towards shaping our direction of travel. The website currently lists 108 digital support offers across the county, which continues to increase (quarterly) due to the work of the network.
- 3.7 Other notable achievements of the DSDN include:

- Over 250 network members.
- The development of a Digital Pantries pilot into 5 DCC supported Community Food Pantries. This integrates basic digital training and support into food pantry settings. 2 further digital pantries are starting in January 2024.
- 5 network events delivered on topics such as digital health apps.
- Planned for Jan 2024 – expansion of PC Refurb work from the High Peak locality to cover the whole DSDN area.
- Planned for Jan 2024 – Digital Awareness Raising Training for organisations across Derbyshire.

Commented [JG(SaT1)]: Months in full please

3.8 On 27 November 2023, the Cabinet Member for Health and Communities was briefed on the Derbyshire Public Health Digital Inclusion Strategy (2023- 2026) and supported implementation being undertaken by officers within the public health team.

3.9 The strategy outlines how Public Health can work with internal County Council departments and other external community stakeholders to champion and shape the future of digital inclusion work across Derbyshire. The three key priority areas are summarised below:

- Residents are empowered to develop and improve digital skills.
- Everyone is connected.
- Digital Inclusion is embedded into organisations and provision is sustainable.

3.10 To kickstart delivery against these priorities this report outlines proposals to strengthen collaboration with VCSE partners, who are best placed to work with local communities to improve digital skills. However, funding for coordination of DSDN is coming to an end, and sustainability of the network is now at risk. Whilst it is recognised that there are over 100 digital support offers in Derbyshire currently, without strategic coordination of the Derbyshire offer there is a risk of duplication across the system. Draft findings from an evaluation conducted by Sheffield University (2023) into DSDN has also concluded that networks “need time to develop, identify synergies and expand their reach”.

3.11 Strengthening future community based digital inclusion is a key enabler for public health and offers the opportunity to contribute towards the following areas of work:

- Addressing the wider determinants of health, e.g., supporting people to gain employment via online applications or by contributing towards income maximisation through online savings.
- Improve efficiency and reduce demand on Council services by supporting residents to identify their own support where possible, or by accessing services more efficiently online through health apps, online appointments, or online information.
- Reduce inequality, by proactively supporting residents at risk of exclusion due to digital by default approaches.
- Addressing community safety concerns through supporting residents to access digital platforms safely and securely.
- Increase the number of digital champions supporting people to improve digital skills across Derbyshire.
- Strengthen digital maturity and understanding of digital barriers across the system.

3.12 Elements outlined above will contribute towards emerging areas of focus in the new Public Health Strategy (2024-2029), recognising that improving digital skills is a key enabler for Derbyshire residents.

3.13 Conversations are ongoing with NHS partners to join up work and support Integrated Care System (ICS) priorities.

3.14 This report also proposes the development of a small grants scheme for DSDN members that will support new services, or the expansion of existing provision into other areas to provide more equitable access. A recent survey conducted with organisations in Derbyshire (n=84) identified the need for increased digital provision, with 41 out of 45 respondents suggesting more funding and financial resources are required to support the continuation of both current and new services that support digital inclusion.

3.15 If approved, administration of the grants will be underpinned by robust criteria and overseen by a funding panel that has representatives from the DSDN and Public Health.

4. Consultation

4.1 In considering the digital needs across Derbyshire, 506 residents participated in a telephone poll. 96 stakeholders from 84 separate

organisations have also played an active role in shaping findings from Citizens Online's research.

5. Alternative Options Considered

5.1 Share Derbyshire's Public Health Digital Inclusion Strategy 2023-2026, without investing any further public health funding into this area of work. This is not a preferred option.

5.2 Allow the DSDN to expire in March 2024, at the end of RAD's funding from NHS Charities Together. This is not a preferred option.

6. Implications

6.1 Appendix 1 sets out the relevant implications considered in the preparation of the report.

7. Background Papers

7.1 Public Health approach to addressing the digital divide. 22 July 2021. Cabinet Member for Health and Communities Report.

8. Appendices

8.1 Appendix 1 – Implications

8.2 Appendix 2 – Digital Exclusion Risk in Derbyshire- Baseline Report: DerbyshireCountyBaselineReportFinal (1).pdf

8.3 Appendix 3 - Derbyshire Residents Phone Poll & overview of findings: Derbyshire Internet Use Phone Poll Findings 2023.pdf

8.4 Appendix 4 – Derbyshire's Public Health Digital Inclusion Strategy: Derbyshire Public Health Digital Inclusion Strategy Document Final 20 Sept 2023.pdf

9. Recommendation(s)



The Commonwealth of Massachusetts
Executive Office of Health and Human Services
Department of Public Health
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DATE: July 21, 2022

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RE: Boston Medical Center — Notice of Transfer of Site Application # BMCHS-22062406-TS

Dear Ms. Harrell,

We are in receipt of the above referenced Notice and its accompanying attachments, dated June 24, 2022, wherein Boston Medical Center (BMC) provided notice to the Department of Public Health (the Department) in accordance with 105 CMR 100.745 for a proposed transfer of site of two outpatient services from various locations adjacent to and within the main hospital as described below:

BMC has complied with the requirements of Notice and has provided information sufficient to allow the Department to make a finding that the proposed transaction will neither result in a Substantial Capital Expenditure nor Substantial Change in Service, and thus will not first require a Notice of Determination of Need (DoN). The proposed transfer of site involves several of BMC's services:

(1) Transfer of Site of outpatient physical therapy (PT) and occupational therapy (OT) services from BMC's main hospital in the Preston Family Building,¹ to a new South Bay Satellite ("Proposed South Bay Project") at 39B District Avenue, Dorchester, MA 02125.

(2) Transfer of Site of outpatient pediatric primary care, pediatric psychiatry, adolescent primary care, and STAR (Services for Trauma and Resources for families) unit services from BMC's main hospital Yawkey Building, pediatric neurology services from BMC's main hospital Shapiro Building,² to BMC's Crosstown Satellite ("Proposed Crosstown Project") located at 801 Massachusetts Avenue, Boston, MA 02118.

Reason for Request

In attempting to meet patient demand for all services, the Applicant has identified a need for strategic space modifications to better use existing building resources, establish centralized services and complementary use adjacencies, and drive operational efficiency. To address the evolving healthcare landscape and to meet

¹ 732 Harrison Avenue, Boston, MA 02118

² 725 Albany Street, Boston, MA 02118,

demand for high quality, cost effective, and efficient patient care, the Applicant states it continually engages in a long-range planning to “right-size its campus.” Through careful site planning of building resource use BMC has undertaken a “measured approach to renovation and new construction” to ensure the Hospital’s sustainability as an academic medical center. The two proposed Transfer of Site Change in Designated Location, Proposed Projects outlined below work towards accomplishing that aim.

Project #1: Proposed South Bay Project

1. ~~Description~~

The Hospital’s outpatient retail pharmacy operates in a leased space;³ that lease will soon end, and thus requires relocation. The pharmacy primarily services outpatients seeking care in the nearby buildings, including the Hospital’s Preston Family (Preston) building. The Applicant has determined the need for the pharmacy to support BMC patients utilizing the outpatient services within Preston. The only space identified was the current location of the outpatient PT and OT services. To accomplish this the Applicant proposes to relocate PT and OT services from the first floor of the Preston Family Building to a new BMC satellite located in leased space at South Bay Center (South Bay), approximately one (1) mile from BMC’s main hospital.

The Applicant asserts that the Proposed South Bay Project constitutes a Transfer of Site or Change of Designated Location of a Health Care Facility, as it involves the relocation of a part of the Hospital from the licensed main hospital to a new hospital satellite.⁴ The Applicant also asserts that the Proposed South Bay Project either when considered alone or when consolidated with the Proposed Crosstown Project (described below) is less than the current expenditure minimum that would require a full DoN submission for either a Substantial Change in Service or Substantial Capital Expenditure. Accordingly, the Applicant seeks approval from the Department to move forward with implementation of the Proposed South Bay Project as a Transfer of Site.

2. ~~Gross Square Feet (GSF)~~

The Hospital’s PT and OT services currently occupy 9,423 GSF within the Preston Family Building. The services will occupy 7.2% more space (10,102 GSF) at South Bay, if approved.

³ In the Doctor’s Office Building

⁴ Pursuant to MGL Ch. 111 s. 25B, “Health Care Facility” is defined to include “a hospital or clinic . . . and any part of such facilities.”

3. Demographics and Primary Service Area (PSA)

The Applicant provided detailed patient demographic and patient origin information of its PT and OT Services.⁵ Given that the South Bay Satellite will be located approximately 1.4 miles from BMC's licensed main hospital, the Applicant does not anticipate any changes to the patient population for BMC's PT and OT services if the project is approved. The Applicant is a high public paying hospital with compensation from government sources exceeding 65%.

4. Patient Access

Given the approaching end of BMC's Doctors Office Building leasehold, the Proposed South Bay Project will ensure continued access to outpatient PT and OT services as well as outpatient pharmacy services for the Hospital's patient panel. Located approximately one (1) mile from BMC's main hospital, the proposed new site is easily accessible by public transportation, is within walking distance of the Andrew Square MBTA Station on the Red Line (0.6 miles), and the Newmarket MBTA Stop on the Fairmount Commuter Rail Line (0.1 miles), as well as several MBTA bus lines and major highways.⁶

Further, the proposed new South Bay site is at ground level, ensuring access for persons with disabilities, is easily navigable, and included free garage parking. Given these factors, the Applicant anticipates that as other services migrate to the Satellite,⁷ the site will allow patients the convenience of an integrated outpatient care setting close to home, transport, and close to BMC's main hospital.

5. Documentation of Sufficient Interest in the Proposed Site

The Hospital has entered into a lease agreement for the proposed new South Bay Satellite. This space is appropriately zoned to provide healthcare services, including PT and OT services. If approved, the Applicant will pursue architectural plan review, custom retrofit the space, and proceed with licensure.

6. Impact on Health Care Spending

Staff will consider this discussion in concert with the second proposed Transfer of site below.

⁵ Which it defines as the cities and towns from which 75% of BMC's PT and OT patients originate.

⁶ major highways

⁷ Separately, the Applicant has signaled the intent to relocate such services as orthopedics to the site. The Applicant is aware that for certain services or Substantial Capital Expenditures, a full DoN submission may be required.

Project #2 Proposed Crosstown Project

1. Description

The Hospital's pediatric primary care, pediatric psychiatry, adolescent primary care, and STAR unit services are located on the sixth floor of BMC's Yawkey Building; its pediatric neurology services are located on the eighth floor of BMC's Shapiro Building. The Applicant proposes to transfer all of these services to the seventh floor of BMC's licensed Crosstown satellite, located across the street, in order to address space constraints driven by increased demand, support a team-based model of integrated pediatric care, and improve use of campus adjacencies, while maximizing the use of existing BMC inpatient and outpatient campus space.

The Proposed Crosstown Project will allow the Hospital to maximize clinical space and redesign patient throughput, thereby reducing the need for travel between BMC's various buildings, improving access for patients. The Applicant anticipates colocation of these services will increase collaboration among providers which is anticipated to improve coordination of care, create efficiencies and facilitate improved health outcomes.

The Applicant asserts that the Proposed Crosstown Project constitutes a Transfer of Site or Change of Designated Location of a Health Care Facility, as it involves the relocation of a part of the Hospital from the licensed main hospital to a hospital satellite.⁸ Additionally, it asserts the Proposed Crosstown Project either when considered alone or when consolidated with the Proposed South Bay Project (described above) is less than the current expenditure minimum that would require a full DoN submission for either a Substantial Change in Service or Substantial Capital Expenditure. Given these factors, the Applicant seeks approval to move forward with implementation of the Proposed Crosstown Project as a Transfer of Site.

2. Gross Square Feet ("GSF")

The Hospital's pediatric primary care, pediatric psychiatry, adolescent primary care, and STAR unit services currently occupy 21,096 GSF within the Yawkey Building, and the Hospital's pediatric neurology services occupy 4,321 GSF within the Shapiro Building, totaling 25,417 GSF. The services will occupy 2.2% more space (25,985 GSF) at Crosstown if approved.

3. Demographics and Primary Service Area ("PSA") Towns Served

⁸ Pursuant to MGL Ch. 111 s. 25B, "Health Care Facility" is defined to include "a hospital or clinic . . . and any part of such facilities."

The Applicant provided detailed patient demographic and patient origin information of its pediatric primary care, pediatric psychiatry, adolescent primary care, STAR unit, and pediatric neurology Services. Given that the Crosstown Satellite is conveniently located across the street from BMC's main hospital, the Applicant does not anticipate any changes to the Patient population served for these services following the Transfer of Site. The Applicant is a high public paying hospital with compensation from government sources exceeding 65%.

4. Patient Access

The Applicant anticipates that the Proposed Crosstown Project will improve access to these services for its pediatric patient population. The proposed Transfer of Site of these services from various buildings within BMC's main hospital across the street to the outpatient Crosstown Satellite will help address care fragmentation and allow patients to receive a full complement of comprehensive, integrated pediatric and adolescent care on a single floor within one building. Such co-location of services will improve the patient care experience. The Applicant anticipates that the Proposed Crosstown Project will reduce the need for travel between BMC's various buildings, increase collaboration among providers, promote better coordination of care, create greater efficiencies in care processes, and, overall, improve access for patients.

5. Documentation of Sufficient Interest in the Proposed Site

The Hospital owns the Crosstown Satellite, which is appropriately zoned to provide healthcare services, including those proposed for this project. Upon approval of the Proposed Project the Applicant will proceed with completion of architectural plan review, renovations, with licensure.

6. Impact on Health Care Spending

Staff will consider this discussion in concert with the first proposed Transfer of site described above.

Overall Impact on Health Care Spending

The Applicant does not anticipate that the either project will have any impact on price, total medical expenditures, or provider costs. As described herein, the Applicant currently provides all these services either at BMC's main hospital or adjacent to it. If approved, all pricing will remain consistent with current charges for these services.

The Applicant states that the combined Capital Expenditures for both proposed transfers of site is \$20,838,426, for the construction/renovation of space to accommodate the projects as shown in Table 1 Below.

Table 1: Cost Breakdown of Each Transfer of Site

Project	#1 South Bay	#2- Crosstown	-
Cost Breakdown	Amount	Amount	Total
Cost of Obtaining Space	\$5,499,224	N/A	\$5,499,224
Construction Contract	\$4,319,084	\$9,731,740	\$14,050,824
Architectural and Engineering	\$388,717	\$874,957	\$1,263,674

The Cabinet Member for Health and Communities approves the award of grants to:

- a) Rural Action Derbyshire (RAD) to coordinate the Digital Support Derbyshire Network (DSDN) for 2 years, subject to satisfactory performance, at a cost of £0.050m per annum and a total value of £0.100m.
- b) Voluntary, Community & Social Enterprise (VCSE) partners to deliver work that strengthens community based digital inclusion. The Digital Support Derbyshire Network will administer the governance, approval and issuing of small grants to community

partners within the network, at a cost of £0.050m per annum for a maximum 2 years at a total value of £0.100m.

Pre- and Post-Filing Planning and Development

Total	\$10,219,140	\$10,619,286	\$20,838,426
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10. Reasons for Recommendation(s)

10.1 Agreeing to the grant funding outlined in this report will enable the DSDN to grow, supporting stakeholders to work in collaboration to improve digital inclusion across Derbyshire.

11. Is it necessary to waive the call-in period?

11.1 No.

Report Authors: Greg Stenson – Service Development Officer, Adam Norris-Service Manager Commissioning, Claire Burnage Public Health Commissioning Officer

SMT Sponsor: Vicky Smyth, Group Manager – Wider Determinants

Appendix 1

Implications

Financial

- 1.1 The £0.100m for the coordination of the DSDN will be funded from the Public Health Reserves and will be issued via a grant agreement in line with Derbyshire County Council's Financial Regulations.
- 1.2 The £0.100m for grants to VCSE partners to deliver work that strengthens community based digital inclusion will be issued by a small grants fund managed by DSDN. This will be funded from the Public Health Reserve and issued via a grant agreement in line with Derbyshire County Council's Financial Regulations.
- 1.3 The Public Health reserve is forecast to be £8.9M at the end of 2023/24 with £3.8m of committed scheme spend approved in 2023-24 to be spent in 2024/25 and onwards. The proposed grant awards can therefore be met from the Public Health reserve balance.

Legal

- 2.1 The Council's Financial Regulations allow grants of up to £0.100m to organisations to be approved by the Cabinet Member.
- 2.2 The Council's standard grant agreement shall be used to set out the terms and conditions for which the grants are made, which provide clawback of funding in certain circumstances and shall also provide that the Council has no liability for any employment liabilities.
- 2.3 The Council has the power to provide funding and grants under the general power of competence set out in section the Localism Act 2011.

Human Resources

- 3.1 The funding will enable local providers to network and provide local digital support to its community. Derbyshire County Council accepts no employment or future redundancy liability, with all employment and related matters to be managed by the providers.

Information Technology

- 4.1 As part of the Council's standard grant agreement, providers will have to complete the Schedule 2 section outlining the measures they will take

around the use of individuals' personal data, to ensure it is processed, stored, and destroyed in line with Council policy.

4.2 There will be no Council IT systems used by providers as part of the DSDN coordination or small grants fund.

Equalities Impact

5.1 The purpose of the DSDN is to connect and empower Derbyshire residents to reduce health inequalities and help tackle financial and social exclusion. Therefore, an Equality Impact Assessment has not been deemed necessary at this time, as there will be sufficient mechanisms within the administration of the two grants to ensure equity of access.

Corporate objectives and priorities for change

6.1 The public health digital inclusion strategy and action plan supports the following DCC priorities:

- (Thriving Communities) Resilient, healthy, and safe communities – enabling residents to manage their health and wellbeing, keep safe online and connecting communities via digital support. It will also support voluntary sector, local networks and libraries strengthen their work in the community, demonstrating the importance of Recommendations 1 and 2.
- (Enterprising Council) High performing, value for money and resident focussed services – improving staff digital skills, transforming inclusivity, improving user journeys and connectivity will enable delivery of better services for less and improve customer satisfaction. Adopting Recommendation 3 will support this process.
- Effective early help for individuals and communities – digital access can support residents to access help and support, also support practitioners to manage risk and models for early intervention. Digital inclusion can enable education, managing mental health, well-being and reduce social isolation. Recommendations 1 and 2 will have a positive impact on identifying needs of residents sooner.
- A prosperous and green Derbyshire - digital inclusion is essential to improve access to better jobs and business growth. Encouraging recycling of tech devices will support the circular economy and reduce

e-waste. Recommendation 3 will support some of these principles of sustainability.

Other (for example, Health and Safety, Environmental Sustainability, Property and Asset Management, Risk Management and Safeguarding)

7.1 In preparing this report the relevance of the following factors has been considered: prevention of crime and disorder, equality of opportunity, environmental, health, property, and transport considerations. There are no anticipated negative impacts from this decision.
~~Project #1: The capital expenditure associated with the proposed South Bay Project is projected to be \$10,219,140. The cost of obtaining the space accounts for the majority of the expenses, 54% and the construction contract comprises the next largest component, 42%.~~

~~The Applicant stresses that for patients, out of pocket costs for patients may be reduced because of access to free parking at the site and that prices will not change.~~

~~Project #2: As shown in Table 1 above, the capital expenditure associated with the Proposed Crosstown Project is anticipated to be \$10,619,286 for renovations to accommodate the relocated pediatric primary care, pediatric psychiatry, pediatric neurology, adolescent primary care, and STAR unit services. The construction contract accounts for 92% of the total costs of this project.~~

~~The Applicant stresses that given current market constraints relating to the inflated costs of construction materials, and architectural and engineering services, the cost of these projects is higher than normal. However, the Applicant notes that the anticipated capital expenditure for each project when considered either individually or aggregated does not exceed the Substantial Capital Expenditure threshold for filing a DoN for outpatient projects, which is currently \$34,988,545.⁹ Accordingly, the Proposed Project does not require a DoN submission.~~

Findings:

~~The Department finds that Boston Medical Center has provided the Department with the information required in 105 CMR 100.745(D). Based upon review of the Notice and supporting materials, the Department finds that this transaction does not require a DoN either as a Substantial Capital Expenditure or Substantial Change in Service, pursuant to 105 CMR 100.730, and that BMC may move forward with the proposed transfers and changes at the designated locations.~~

~~[signature on file]~~

~~Margret R. Cooke~~

⁹~~For FY 2022, it is adjusted annually for inflation.~~

Commissioner